

1. Administrative & Study Identification

Full Study Title: A Study of Relatlimab Plus Nivolumab Versus Nivolumab Alone in Participants With Advanced Melanoma **Short Title/Acronym:** RELATIVITY-047 **Protocol Number:** CA224047 **NCT Number:** NCT03470922 **Posting ID:** 679471553948741389 **Sponsor Name:** Bristol Myers Squibb **Investigational Product:** Opdualag (relatlimab and nivolumab fixed-dose combination) vs. Opdivo (nivolumab) **Phase of Development:** Phase II/III (PHASE2, PHASE3) **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Medical Monitor:** Bristol-Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To determine whether relatlimab in combination with nivolumab is more effective than nivolumab monotherapy in improving Progression-Free Survival (PFS) in patients with previously untreated metastatic or unresectable melanoma. **Secondary Objectives:** To evaluate Overall Survival (OS) and Overall Response Rate (ORR) in the study population.

2.2 Background & Rationale To address the need for improved first-line therapies in advanced melanoma. The combination introduces a novel approach using two immunotherapies that act together to enhance anti-tumor response by simultaneously targeting two distinct immune checkpoints: LAG-3 (targeted by relatlimab) and PD-1 (targeted by nivolumab), aiming to overcome immune exhaustion and improve outcomes over PD-1 inhibitor monotherapy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Nivolumab monotherapy) **Blinding:** Double-blind **Randomization:** 1:1

3.2 Planned Enrollment & Duration Target \$N\$: 714 **Number of Sites:** Global (approximately 127 locations) **Estimated Study Start:** 11-Apr-2018 (Actual) **Estimated Primary Completion:** 25-Jan-2021 (Actual)

4. Subject Selection (Eligibility Criteria)

For more information regarding Bristol-Myers Squibb Clinical Trial participation, please visit www.BMSStudyConnect.com

4.1 Inclusion Criteria 1. Age ≥ 12 years. 2. Histologically confirmed Stage III (unresectable) or Stage IV melanoma, per the AJCC staging system. 3. ECOG Performance Status of 0 or 1. 4. Tumor tissue from an unresectable or metastatic site of disease must be provided for biomarker analyses. 5. Participants must not have had prior systemic anticancer therapy for unresectable or metastatic melanoma.

4.2 Exclusion Criteria 1. Active brain metastases or leptomeningeal metastases. 2. Active, known, or suspected autoimmune disease. 3. Medical conditions requiring systemic treatment with moderate or high-dose corticosteroids or immunosuppressive medications. 4. Diagnosis of uveal melanoma. *(Other protocol defined inclusion/exclusion criteria could apply)*

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: - Arm A (Experimental): Fixed-dose combination of 160 mg relatlimab and 480 mg nivolumab (Opdualag) every 4 weeks. - Arm B (Active Comparator): 480 mg nivolumab (Opdivo) alone every 4 weeks. **Route of Administration:** Intravenous (IV) infusion. **ATC Code:** L01FF01 (Nivolumab) **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care. **Prohibited:** Systemic treatment with moderate or high-dose corticosteroids or immunosuppressive medications; concurrent investigational treatments.

5.3 Product Information References (EMA) - Opdivo: [EMA Product Information](#) - **Opdualag:** [EMA Product Information](#)

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Tissue Biomarker Analysis, Baseline Scans
Baseline	Day 0	Randomization (1:1), First IV Infusion
Interim	Every 4 Weeks (Q4W)	IV Infusion, Safety Labs, Efficacy Assessment (RECIST v1.1 Scans), AE Monitoring
End of Study	Follow-up	Final Vital Signs, Exit Interview, Survival Follow-up after Disease Progression or Unacceptable Toxicity

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS), defined as the time between the date of randomization and the date of first documented tumor progression, assessed by a blinded independent central review (BICR) (per RECIST v1.1 criteria), or death due to any cause, whichever occurs first. Subjects who die without a reported progression will be considered to have progressed on the date of their death.

7.2 Secondary & Exploratory Endpoints - Overall Survival (OS): Time between the date of randomization and the date of death due to any cause. - Overall Response Rate (ORR): Number of randomized subjects achieving a best response of Complete Response (CR) or Partial Response (PR) based on BICR. - Melanoma-Specific Survival (MSS).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via onsite visits and routine clinical laboratory evaluations; special focus on immune-mediated AEs. **Serious Adverse Events (SAEs):** Monitoring for grade 3/4 treatment-related adverse events (TRAEs), which include musculoskeletal pain, fatigue, rash, pruritus, and diarrhea. **Data Monitoring Committee (DMC):** External independent Data Monitoring Committee for safety and efficacy oversight.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all randomized patients to evaluate efficacy endpoints. **Sample Size Justification:** $N = 714$ appropriately powered to detect statistically significant improvements in PFS and OS. **Handling of Missing Data:** Patients alive without progression at data cutoff are censored at the date of last contact ("last known alive date").

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring of clinical sites to ensure adherence to the protocol and accurate reporting of RECIST criteria. **Audit Trail:** eCRF locking, tracking of treatment adherence, and data clarification forms via standard clinical audit mechanisms.

11. Study Identifiers & Governance

Primary ID: CA224047 **Posting ID:** 679471553948741389 **Registry Number:** NCT03470922 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** RELATIVITY-047 **Official Regulatory Title:** A Randomized, Double-Blind Phase 2/3 Study of Relatlimab Combined With Nivolumab Versus Nivolumab in Participants With Previously Untreated Metastatic or Unresectable Melanoma

12. Clinical Framework (Melanoma Specific)

Primary Condition: Malignant Melanoma / Advanced Melanoma
Semantic Type: Neoplastic Process **Diagnosis Definition:** A malignant neoplasm derived from cells that are capable of forming melanin, which may occur in the skin of any part of the body, in the eye, or, rarely, in the mucous membranes of the genitalia, anus, oral cavity, or other sites. It occurs mostly in adults and may originate de novo or from a pigmented nevus or malignant lentigo. Melanomas frequently metastasize widely, and the regional lymph nodes, liver, lungs, and brain are likely to be involved. **Reference Ontology Codes:** - **MeSH Code:** D008545 - **SNOMED ID:** 372244006 - **HPO Code:** HP:0002861 **Diagnosis Threshold:** Histologically confirmed Stage III or IV melanoma per AJCC staging **Medical Methodology:** Dual Immune Checkpoint Inhibitor Therapy **Optimization Target:** LAG-3 and PD-1 checkpoint inhibition

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for advanced melanoma including routine RECIST v1.1 imaging. **Education Protocol (HCP):** Site initiation visits and ongoing training on handling immune-mediated adverse events. **Education Protocol (Patient):** Informed consent process, counseling on potential treatment-related side effects (e.g., rash, diarrhea). **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for all radiological endpoints to prevent investigator bias.

14. Observation & Follow-Up Schedule

Total Duration: Treatment until disease progression or unacceptable toxicity; long-term survival follow-up up to estimated study completion in Dec 2030. **Onsite Visit Cadence:** Every 4 weeks (Q4W) for treatment infusions. **Remote Monitoring Frequency:** Intermittent survival tracking post-treatment discontinuation. **Checklist Review Interval:** Regular tumor imaging according to protocol-specified RECIST interval (typically every 8-12 weeks).

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					